A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

20 May 2026

Dr Phillip Boltin
Betta Health Medical Centre

Dear Dr Boltin,

Re: Ms Marree Brown
DOB: 19/03/1966 Phone: 0408057173
Address: 82 Brunswick Road, BRUNSWICK, VIC, 3056
Email: marreebrown@iprimus.com.au

Summary

- **Complete intestinal metaplasia (low-risk) in the antrum.**
- **Endoscopic findings of autoimmune gastritis and confirmed pernicious anaemia.**
- **Surveillance gastroscopy recommended in four years.**
- **Small bowel capsule endoscopy deferred pending follow-up iron studies.**
- **Action requested Please repeat iron studies in four to six months.**

I had the pleasure of reviewing Marree Brown in the clinic to discuss the results of her recent gastroscopy. I am pleased to report that we found no evidence of malignancy, and the biopsies from her small intestine were normal, which effectively rules out malabsorption at that site as a cause for her iron deficiency.

There were, however, two significant findings in the stomach. Firstly, the biopsies confirmed the presence of complete intestinal metaplasia in the antrum. This is a low-risk, complete subtype and appears localised to the bottom of the stomach. We also confirmed that she is H. pylori negative. Secondly, the gastric lining appeared endoscopically flat and atrophic. While the biopsies themselves were not definitive for autoimmune gastritis, the visual appearance, combined with her clinical history, is classic for this condition.

Marree has a known history of B12 deficiency requiring injections, and a strong family history of the same. Given the endoscopic appearance of gastric atrophy, I have confirmed a diagnosis of pernicious anaemia secondary to autoimmune gastritis. This explains her B12 requirements, though it does not fully account for her iron deficiency.

Impression and Plan Regarding her risk of gastric malignancy, both the autoimmune gastritis and the intestinal metaplasia warrant long-term monitoring. While some guidelines for low-risk metaplasia in Caucasian patients suggest surveillance may not be mandatory, the combination of findings here makes a conservative approach prudent. We have agreed on a surveillance gastroscopy in four years, or sooner should she develop red-flag symptoms such as early satiety, vomiting, or haematemesis. Regarding her iron deficiency, this has been a very slow decline over the five years since we first met, which is reassuring against a sinister cause. I have suggested that you repeat her iron studies in four to six months. If her iron levels continue to decline or drop rapidly, I have recommended we proceed with a small bowel capsule endoscopy to evaluate the mid-GI tract. Marree is happy to wait for these blood results before committing to

All appointments and enquires
Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
Ph: 03 9650 7917
Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

further investigations.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

Investigation Results:

BROWN, MARREE : SVH 1305437

DOB: 19/03/1966 Age: 60 years

82 Brunswick Road, Brunswick 3056

Results from 28/04/2026 ..... Episode# 26P306692 Test Hist (Histology - Routine)

FINAL ANATOMICAL PATHOLOGY AAnat

Lab #: 2026B011017 **CLINICAL NOTES** Iron deficiency. Atrophy gastritis. ? Antral IM. ? H. pylori.? Autoimmune. **MACROSCOPIC DESCRIPTION** 1. Specimen labelled "Mid Oesophagus". Two mucosal fragments measuring 1-3 mm. A1. 2. Specimen labelled "Body". Four mucosal fragments measuring 1-4 mm. A1. 3. Specimen labelled "Antrum". Three mucosal fragments measuring 1-5 mm. A1. 4. Specimen labelled "D2". Four mucosal fragments measuring 1-2 mm. A1. **MICROSCOPIC DESCRIPTION And DIAGNOSIS** **1. Mid oesophagus - Two strips of mildly inflamed squamous mucosal tissue with presence of intraepithelial lymphocytes. No eosinophils are seen. No fungal elements are seen. No fungal elements are seen. The features are consistent with reflux oesophagitis. No evidence of dysplasia or malignancy. 2. & 3. Body and Antrum - Both specimens show a similar appearance. Morphological features of chronic inactive gastritis of chemical/reactive type with complete intestinal metaplasia.. No Helicobacter pylori-like organisms are seen. No evidence of dysplasia or malignancy. 4. D2 - Duodenal mucosal biopsies with no significant pathological features. Morphological features of coeliac disease are not seen. No granulomas or parasites are identified. No evidence of dysplasia or malignancy.** Pathologist: Dr Cesar Salinas-LaRosa SNOMED T#: 63

All appointments and enquires
Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
Ph: 03 9650 7917
Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065